

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

**MARIA GEORGE AND CRYSTAL
MARIA GEORGE & WILSON LARRY
GEORGE**

Plaintiffs

- and -

**COVENT GARDEN MARKET
CORPORATION**

Defendant

)
)
) Vicki J. Edgar, for the Plaintiffs
)
)
)
)

)
)
)
) Geoffrey P. Belch, Assistant City Solicitor
) for the Defendant
)
)
)

) **HEARD:** June 8, 11, 12, 13, 2007

JUDGMENT

ROSS J.

[1] The plaintiff Maria George seeks non pecuniary damages as the result of a trip and fall on the defendant's premises in the City of London. The other two plaintiffs, Crystal George, Maria George's daughter and Maria George's husband, Wilson George, seek damages under the *Family Law Act* for services rendered to the plaintiff as a result of her fall and for loss of guidance, care and companionship in respect of Maria George. For convenience, Maria George will be referred to as the plaintiff.

[2] The defendant is an independent corporation. The Covent Garden Market is a building located in downtown London and sits on land owned by the city. It is a modern day farmer's type market with approximately fifty-two independent retailer within the market. The main floor consists of several types of retail vendors offering produce, poultry, meats, baked goods and florists. There are also eating areas and a restaurant. The second floor or mezzanine provides space for cultural organizations and artists along with an art school and a theatre. The market opened in October 2000 and has approximately 4000 visitors per day. There are three main entrances and a smaller entrance from the restaurant. The location of the plaintiff's fall was at the north entrance which faces onto a street named Covent Market Place and also Market Lane. There is a double set of glass and metal doors with a vestibule between the two sets of doors. Both sets of doors swing inward and are wheelchair accessible. The plaintiff fell inside the vestibule area.

[3] Immediately inside the outer doors is an area of smooth surface concrete flooring. No dimensions were given in relation to the concrete floor of the vestibule or the metal grate in the floor. I can only estimate from the photos entered at trial that the floor area extends about two feet or so inward and then there is a rectangular metal grate imbedded in the floor. Again I can only estimate it to be about six to seven feet square as I view it in the photographs. Beyond the grate is again the concrete floor. I estimate the second set of glass metal doors is about two feet or so from the edge of the metal grate or about the same distance as the outer doors to the metal grate.

[4] The plaintiff's fall occurred on February 7, 2002, around twelve thirty p.m. On that day, in accordance with the usual practice of the defendant during the winter months, there was a commercial grade mat measuring eight by four feet placed in the vestibule. There was a second mat of similar size inside the market immediately inside the interior set of doors. The mat in the vestibule, as shown in the photographs, was laid lengthwise between the two sets of doors and more or less in the centre area of the metal grate. The width of the mat did not cover the width of the grate. Portions of the grate are exposed on each side of the mat. The leading edge of the mat as one enters through outside doors is a few inches inside the grate area. Again from the photographs, I estimate the edge of the mat is less

than a foot from the edge of the grate. At the other end the mat proceeds beyond the edge of the grate and onto the concrete floor and ends close to the inside set of doors.

[5] The day was sunny. There was no ice and the snow was melting. The ground was wet with some dry patches. The sidewalk area at the north doors had been cleared of ice and snow. The plaintiff entered through the first set of doors. She said the floor was relatively dry and there were no puddles of water. Ms. Karin Hanwell, the assistant manager at the market, said that the north entrance area was dry and there was no debris on the floor.

[6] The plaintiff describes the mat in the vestibule lying at an angle rather than perpendicular between the two sets of doors. She describes there being two people standing inside the first set of doors to her left. They were not standing on the mat. The plaintiff says that she had to go around them when entering the vestibule. She took about six steps inside the door and then two steps onto the mat. At that point she observed a buckle or roll in the mat which she places toward the far end of the mat and about two feet or so from the inside doors. She describe the buckle, roll or raised up area across the mat being about six inches high on the left side of the mat and tapering down toward the right side. Although having seen the buckle in the mat she said she could not avoid it and believes her right foot caught on this buckled area causing her to fall forward with considerable force into the left side of the door frame of the second set of doors. She raised her arms to protect herself as she fell into the door frame.

[7] The plaintiff said that she struck the left side her head on the door frame and then rebounded backward falling on her back and hitting her head on the floor. She also said that it was possible she had fallen backward on her side and hit her head on the floor. Although not sure, she thought the left inner door had swung back and hit her in the head. The ambulance report notes her stating that her falling against the inner door caused it to open and that it hit her again as it closed. The plaintiff said that after hitting her head she was dazed. The ambulance report notes complaints of pain in her head, neck, both arms and both knees. She was taken by ambulance to the London Health Sciences Centre, (LHSC).

X-rays of her neck showed a localized prevertebral soft tissue swelling at the C5-6 level. There were no fractures. Her left wrist was x-rayed and showed a mild soft tissue swelling over the ulnar side of the wrist, with no fractures or dislocations. The plaintiff was released from the hospital the same day.

[8] The market has security video cameras. One is located inside the market and focuses on the north side entrance. It is not of the highest quality in definition but does show the outside and inside doors and the vestibule. The mat is visible in the video. The video was played during the trial and I have since had occasion to review it. The plaintiff and the defence witnesses had seen the video prior to trial. There was also an independent witness, Susan Sutherland, who observed the plaintiff's fall.

[9] Ms. Susan Sutherland is part of the clerical staff at Bell Canada and has worked there for about 31 years. She attended the north entrance of the market to access the TD Bank automatic teller to get some money for her lunch. The automatic teller is located in the vestibule of the north entrance. On entering the vestibule the machine is to the right of the grate and backs against the inner wall in which the second set of doors is located. Immediately next to the machine is a post housing an access button to operate the inner doors for wheelchair accessibility. The bank machine and post do not interfere with passage through the vestibule between the two sets of doors.

[10] Ms. Sutherland said there were two people in line at the teller machine. She stood aside and away from the machine at a railing located at the other side of the mat and grate, so that the mat was between her and the bank machine. Ms. Sutherland was alone. She observed the plaintiff enter through the right side door of the outer doors and trip in the area of the beginning of the mat. She described the plaintiff pitching forward and flying into the left side of the inner door frame with significant force. She said that the inner door was closed and that the plaintiff hit the left side door frame and sort of crumpled face forward down on to the mat. She circled on one of the photographs the area where the plaintiff tripped. That location being between the end of the concrete floor across the small

area of metal grate and the beginning of the mat. She was quite clear the plaintiff tripped in that area, at the beginning or around the leading edge of the mat and not further along the mat as described by the plaintiff. She also stated that she did not see any buckle or roll in the carpet further along its length, as described by the plaintiff. In cross-examination, Ms. Sutherland was shown one of the photographs on which the plaintiff had marked the location of the buckle in the mat and the place where she tripped. Ms. Sutherland was unshaken in her testimony that the plaintiff did not trip in that area, that is, further along the mat. She said the plaintiff tripped at the beginning of the mat.

[11] Ms. Sutherland completed her banking, spoke with a security guard at the market, gave her name and wrote out on an accident report type form a brief description of what she had seen, namely, “came through outside door tripped on black mat went flying into metal door frame”. Ms. Sutherland estimated that she was in the vestibule area less than ten minutes and left that area prior to the ambulance paramedics’ arrival.

[12] Karin Hanwell as assistant manager of the market was responsible for the maintenance of the public areas of the market. The maintenance staff, who are referred as “Market Hosts” reported to her. From the times shown on the video and the still photograph reproduced from it, the plaintiff fell between 12:35:13 and 12:35:15. Ms. Hanwell is seen in a video photograph at 12:37:24, or about two minutes after the plaintiff fell. She remained with the plaintiff until the plaintiff was removed by the paramedics. She held the plaintiff’s head and told her to stay calm. She describes the plaintiff as very upset and saying “it hurts every where.” She asked the plaintiff where it hurt and again the plaintiff’s response was “every where”. Ms. Hanwell in an attempt to localize where the plaintiff hurt asked if her head hurt, if her neck, legs, arms hurt etc.. She said the plaintiff responded with an emphatic “yes” to each of her questions. She describes the plaintiff as very agitated, frantic, and in Ms. Hanwell’s opinion the plaintiff was over reacting. She described the plaintiff as being “over the top”. According to Ms. Hanwell the paramedic team had difficulty with the plaintiff who would not let them touch her. She said that finally one of the paramedics told her they would have to touch her. The plaintiff had to be placed on a back board and then lifted up onto a gurney. The emergency room report refers

to the plaintiff yelling at the staff. The plaintiff explained that her demeanour was because of the pain she was experiencing from her injuries.

[13] Ms. Hanwell had to fill in liability claims report for her employer's insurance administrators. She asked the plaintiff what happened. She quote the plaintiff first saying, "I don't know, I don't know. I might have tripped on the carpet". Subsequently, the plaintiff told her that she had "tripped on a rolled carpet". Ms. Hanwell recorded what the plaintiff said on the report. Ms. Hanwell made it quite clear that what she wrote on the report was what the plaintiff told her and not what she herself had observed.

[14] Ms. Hanwell said that upon her arrival she did not straighten the mat as it was not necessary to do so. She said there was no roll or buckle in the mat and had there been one she would have leveled the mat as customers were still entering and exiting through that part of the vestibule. She described the weather as clear and not wet and that there was no water or debris in the entrance way. Shortly after Ms. Hanwell arrived, Mr. Christopher Testolini, one of the maintenance staff, arrived and locked the outside doors.

[15] I viewed the video when it was seen during the plaintiff's cross-examination at trial and have reviewed the video tape since the trial. There are two time counters show on the video which are slightly different in time the one from the other. The times I shall refer to are the time shown with the larger numbers. The tape commences at 12:24:47 p.m. The plaintiff identifies herself in a frame at 12:35:13. She appears to be located on or at the beginning of the mat. The next frame is at 12:35:15 wherein she is seen lying in front of the inner doors. The frames are not continuous. There is a two second gap between frames. During the two second gap between those two frames the plaintiff fell, accordingly, there is no actual frame showing the fall.

[16] There is about ten minutes of video from the commencement of the tape up to the frame first showing the plaintiff at 12:35:13. During those ten minutes the video show numerous people traversing through the vestibule and walking the length of the mat and also across the mat with no apparent difficulty. The mat appears to remain stationary during that time. Three frame starting at 12:35:05 to 12:35:09 show a person approaching the outer

doors, then walking at the middle of the mat and then slightly inside the market . Those frames indicate that the person walked the length of the mat without difficulty. The next frame, two seconds later, show no one walking or standing on the mat. Two seconds later at 12:3513, the plaintiff is first see in the area at the beginning of the mat. From the foregoing, the video displays that four seconds before the plaintiff stepped on the mat, another person had walked the length of the mat without mishap.

[17] The video continues on for a time. There is then a new and later sequence showing the paramedics attending to the plaintiff who was still on the floor. At 13:03:50 the video frame and a video photograph of the frame show a buckle in the mat which appears to be around the area of the edge of the grate closest to the inner doors. From the ambulance report the paramedics arrived at the plaintiff's side at 12:52. Allowing that there are two times shown on the video, I estimate that the paramedics were attending to the plaintiff for about ten minutes before the time that the buckle in the mat appears in the video. The plaintiff identified this as the buckle in the mat which caused her to trip and fall. The scene is viewed from inside the market looking to the outside and only part of the mat is visible with part obscured from view by the gurney and the paramedics. The plaintiff said that the buckle as seen on the left side of the photograph was lesser in height than it was on the other side of the mat, which is obscured in the photo. On her discovery she had said that the buckle was only halfway across the mat. In relation to this photograph and her evidence on discovery, the buckle would have been only in the obscured area of the photograph. This video photo is timed at 13:03:50; about 28 minutes after the plaintiff fell. The immediately preceding frame, two seconds earlier at 13:03:48, shows the same area of the mat with what appears to be the beginning of the buckling in the mat. The video shows the three paramedics moving around the plaintiff while on the mat. They had to place a back board under the plaintiff in order lift her onto the gurney, and as earlier mentioned the plaintiff was a somewhat uncooperative patient. In my opinion the buckle referred to and identified by the plaintiff as the buckle which caused her to trip was probably caused by the movement of the paramedics on the mat in attending to the plaintiff.

[18] In her examination in chief the plaintiff stated that on February 7, 2002 she had no problem with her vision, legs, balance and no dizziness. As adduced in her cross-examination, the plaintiff has a history of falls prior to February 7, 2002. In her youth she was knocked off her bicycle and fell on her right knee on a broken bottle causing injury to that knee. In April 1993 the plaintiff fell on her right knee in the Oxbury Mall in London. That fall resulted in the plaintiff having arthroscopic surgery to that knee. In March 2001 the plaintiff fell while shoveling snow in her driveway. She fell on her back and was not able to get up for half an hour. She explained it was icy preventing her from getting up. Around November 1, 2001 the plaintiff slipped on the floor of a city bus. She grabbed hold of a bar and did not fall completely, however, her legs splayed and she injured her left knee. There is a notation in her family doctors' file dated January 14, 2002.

“Patient’s visit to-day about fall from bus early November 2001. Have (sic) been seen by Dr. Kamath Nov. 13. Pain in left inner leg. Intermittent effecting ambulation and bending of knee without swelling. Patient denies redness.

Complaint of pain, medial side of left knee since fall on Nov. /01 (without swelling) occasionally while walking she gets a sharp pain. Sometimes she wakes up with severe pain in the knee and relates it to kicking around in the night (because) of her sleep apnea and then has to hobble around on her cane.

[19] The plaintiff was sent for an x-ray of both of her knees, which was performed on January 18, 2002, approximately three weeks before her fall at the market. The report reads

Left knee pain increasing since fall November 1.

Right knee shows ossification at the origin of the medial collateral ligament. This would be consistent with an old injury. The knee was otherwise unremarkable.

The left knee shows no definite abnormality. There is a bipartite fabella.

[20] Ms. Sutherland evidence was that the plaintiff tripped at the beginning of the mat. There was no evidence that the leading edge of the mat was curled up, such as might catch someone's foot. The video and the photographs taken from it show the leading edge of the mat where Ms. Sutherland saw the plaintiff trip lying flat on the metal grate.

[21] Karin Hanwell, along with Christopher Testolini and Edward Michael Burgess who were two of the maintenance staff on February 7, 2007, gave evidence in regard to the manner in which the public areas of the market are maintained. The tenants of the market are responsible to look after their own space. Ms Hanwell was responsible for the public areas and the maintenance staff, of which there are usually two or three. She had been in the commercial hospitality industry for twenty-two years. She commenced her employment at the market in October 2000 and left that job in August 2003, taking a position in the TV media. The market opens at 8:00 a.m. and closes at 6 p.m. Ms. Hansen's hours were 9:00 a.m. to 6:00 p.m. The morning maintenance staff starts at 6:30 a.m. The second shift starts later and works till after the market is closed. There is also a night maintenance man. Ms. Hanwell's office was on the main floor. She had radio contact with the maintenance staff and she spent about twenty-five percent of her time in the office and seventy-five percent out in the public areas dealing with the maintenance people. When out in the public areas walking around she would check things.

[22] There were two check lists required to be completed by the staff. One was the early morning check list. That had to be completed before the market opened in the morning. There was also a closing check list which had to be completed relating to items that were to be attended to after the market closed. On February 7, 2002 the early morning list was marked with all listed items check as done. It was signed by Norman Lavigne. The evening check list was completed by Mr. Burgess showing all items had been addressed. The morning list does not specifically note mats or grates or the vestibules, but does refer among a number of other things, for example, whether the floors were clean and in good repair: whether all doors/exits were free of obstruction and whether the handicapped access was working condition and the access clear. The north entrance was handicapped accessible so that area would have been included in the morning inspection.

[23] Ms. Hanwell addressed the issue of the mat. She stated that the mats used were industry standard, which I interpret to mean that they are a type of mat that is standard in a commercial undertaking such as the market. They are used during the winter and are replaced with fresh mats weekly. She said they adhere well to the floor. The mats are supplied by an independent contractor. In the summer they are used for wet weather. If the mats were wet there are hung up to dry during the night. The staff determined if that was necessary.

[24] Ms. Hanwell testified that prior to the plaintiff's fall she had never had any complaint about the mats and on the day of the fall she had received no prior complaints about the mats at the north entrance. Ms. Hanwell said she didn't recall the grates in particular and did not comment on them.

[25] Mr. Testolini worked at the market for three years and left in 2003. He was working in maintenance on February 7, 2002 and was on duty when the plaintiff fell. He started at 6:30 that morning and was working with another maintenance man. He had an hour and a half to do the pre-opening preparations of the building. He said because the over night maintenance man was excellent that there wasn't much to do in the morning other than touch up work. He said the routine was to start at the east side of the building with the garbage room, walk up through the food court, look in the bathrooms and then head to the vestibules. He would proceed up stairs to check the mezzanine. Garbage receptacles were checked to see if they needed emptying. He said the process of going around and checking things was continual. He commented that the check lists was a method of communication between the shifts and was used to let the incoming shift know if something had not yet been done. He said the lists are misleading in that it was not just a matter of doing what was on the list and the work was finished. He said the work was continuous. He stated that cleaning the vestibules, which included the vestibule at the north entrance where the plaintiff fell, the windows and maintaining the garbage was a continual thing and not something which was done once in a while. He said the mats were looked at during his walking around. He explained that the mats can get very wet from patrons tracking in snow from the outside, particularly the mat inside the market. They would rotate the wet inside mat with the mat in

the vestibule and put the wet mat on the grate to drip dry and bring the outside mat inside the market.

[26] Mr. Belch, defendant's counsel, directed Mr. Tortilini to the plaintiff's complaint that she tripped on a buckle or roll in the mat. Referring to the plaintiff's law suit he said, "...this case is pretty unique...I don't believe we've had any incidents regarding people tripping over a mat."

[27] Mr. Testolini was asked about the grate in the north entrance vestibule. He stated that one of the staff's duties was to keep the vestibules clear of debris and dry. He said that it could get very slippery in the vestibules and that was one of the reasons for putting mats over the grates, because if the grate was wet it would become a skating rink. He explained that the market had a lot of business woman coming into the premises and that it would be a challenge for a woman in high heels to negotiate the grate when it's slippery. Accordingly, the mats were laid over the grates as a safety measure.

[28] In cross-examination Mr. Testolini said that the grates which are screwed into the floor were smooth with slits or openings between the metal strips to allow water to drip through. The grates are bordered by a collar of cement. He was asked if this cement edging was raised above the grates. He said "Yeah, pretty flush, but slightly." The following questions and answer relate to the mat and the grate.

- Q. And would you agree with me that a mat that has rubber backing that's made to stick to concrete flooring will slide a little bit more on metal?
- A. ... it would be if the entire mat was on that grate, but as you can see, the mats are actually overlapping or on the concrete as well.
- Q. But that part that's stuck to the grate will actually slip a bit more?
- A. That hasn't been my experience with those mats, no.

[29] In re-examination Mr. Tortolini was asked specifically asked what was done throughout the day in regard to the vestibules. His response was as follows.

Um, they were, um, depending on the weather also, but we would clean windows and mop the vestibules. For a day like February, you know, if there was excess moisture, we would actually grab a mop bucket and make sure that it was clear of excess water, and sweeping up any debris that would gather in the vestibules over the course of a day.

[30] The following questions and answers relate to his routine when touring the market during the day.

A. Well, it was fairly regular and as we set into a routine, it wasn't a routine that we had to follow per se, but it was one that grew as the job grew, it made common sense to do this routine for the market, it was check the garbage room, go to the food court, check the bathrooms, check the vestibules, check the stairwells, mezzanine level, come back down and do the whole thing again, so that was basically the routine that I fell into.

Q. So there was a routine that was followed?

A. Yeah.

Q. Just not as specific at the time that it had to be done?

A. Exactly, exactly.

[31] Mr. Burgess was part of the maintenance staff and had started his job about a year and a half before the plaintiff's fall. On February 7 he started in the afternoon and was not on duty at the time the plaintiff fell. He described the maintenance routine of the market as follows;

. Generally, with that, for morning opening up, we'd go around, check all the floors, check all the entrance ways for debris, garbage, make sure the mats were out and cleaned as well from the night shift, and just walk around and make sure that everything was in order for opening, and cleaned up and ready for opening.

[32] Mr. Burgess stated that during the winter months mats were used all the time. He was asked if there were any problems he had ever heard about or complaints made about the mats buckling. He replied, "no, not while I was there, no." He also stated, in respect of water that

would be tracked in by patrons, that if the mats were excessively wet, the staff would place an additional mat inside the market entrance to catch the extra water and if a mat was very wet it would be replaced with a fresh mat from the extra mats they had on hand.

[33] In regard to the mats being placed on the grates he stated;

I believe it was a safety feature, because as you see, at the entrance, there were steel grates in the foyer there, therefore, once ice and water and snow would get on them, then they'd be like a skating rink, so we put the mats down to protect, and for safety reasons.

Law

[34] Section 3 (1) of the *Occupiers Liability Act* provides that,

An occupier of premises owes a duty to take such care as in all the circumstances of the case is reasonable to see that persons entering on the premises, and the property brought on the premises by those persons are reasonably safe while on the premises.

[35] In *Garofalo v. Canada Safeway Ltd*, [1998] O.J. No.302, Kozak J. reviewed the case law which interpreted the above section of the *Occupiers Liability Act*. The following paragraphs taken from Kozak J.'s reasons set out the principles which must guide the court in this case.

19 In the case of *Waldick v. Malcolm et al* 70 O.R. (2d) 717 Ontario Court of Appeal, it is stated at page 723:

All Courts have agreed that the section imposes on occupiers an affirmative duty to make the premises reasonably safe for persons entering them, by taking reasonable care to protect such persons from foreseeable harm. The section assimilates occupiers liability with the modern law of negligence. The duty is not absolute and occupiers are not insurers liable for any damages suffered by persons entering their premises. Their responsibility is only to take such care as in all the circumstances of the case is reasonable.

¶ 20 The Supreme Court of Canada in *Waldick* (1991) 83 D.L.R. (4th) at page 18 stated that:

The goals of the Act are to promote and indeed require where circumstances warrant, positive action on the part of occupiers to make their premises reasonably safe.

22 The factors which are relevant to an assessment of what constitutes reasonable care will necessarily be very specific to each fact situation ... thus the proviso "such care as in all the circumstances is reasonable". (*Waldick Supra* p. 13)

27 At page 8 of *Hoffert Bouck J* goes on to state:

The plaintiff must prove the conduct of Westfair was unreasonable in the circumstances on a balance of probabilities. If I were to hold the defendant acted unreasonably in these circumstances, that would mean it would have to schedule the cleaning of the grape table area more often than every twenty-five minutes the store was open. But even that might not be sufficient to prevent harm. The law does not demand a system of constant surveillance and instant response. (*Beaman v. Canada Safeway Ltd.* (1993) Sask. Q.B. November 12, 1993 No. 3377 Lawton J.)

¶ 28 The duty of care does not extend so far as to require the defendant to remove every possibility of danger. The test is one of reasonableness and not perfection. (*Carlson v. Canada Safeway Limited* 47 B.C.L.R. 252 B.C.C.A.; *Gerak v. R. in Right of B.C.* (1984) 59 B.C.L.R. 273 (C/A).

¶ 29 Tort law involves the attribution of the risk or the allocation of the responsibility for the accident. In this case we are dealing with a statutory duty of care imposed by Section 3 of the Occupiers Liability Act which requires that an occupier take such care as in all the circumstances of the case is reasonable to see that persons entering on the premises are reasonably safe when on the premises. At the same time, a person coming onto the property is expected to be reasonably alert to any obvious dangers (i.e. not be contributorily negligent) and not voluntarily assume the risk.

¶ 30 The onus is on the plaintiff to prove on a balance of probabilities that the defendant was in breach of a positive duty of care. She must be able to point to some act or omission on the part of the defendant which resulted in her injury. In this case I take it that she relies upon the failure of the defendant to pick up a piece of plum which was apparently discarded in front of the bin in photo B as being the cause of her injury.

¶ 31 The positive or affirmative duty that is imposed upon the defendant does not extend to the removal of every possible danger. It does not require the defendant to maintain a constant surveillance or lookout for potential danger. The defendant meets its duty to take reasonable care if it takes measures that are reasonable in the circumstances.

[36] Kozak J's conclusions in *Garofalo* set out in the above quoted paragraphs 29,30 and 31, were adopted by Cavarzan J in *Walton v. Plainsman Dinning Rooms Ltd*, [2004] O.J. No.589.

[37] Ms. Edgar, plaintiff's counsel, argued the issue of liability on two different factual bases. She argued that it was clear that the plaintiff fell. While noting the conflict in the evidence between the plaintiff and Ms. Sutherland about whether there was a buckle in the mat, she referred to the photographs taken from the video showing a buckle in the mat while the paramedics were attending to the plaintiff. She argued that the buckling seen at that time indicates that the mat was prone to buckling and supports the plaintiff's position that she tripped on a buckle in the mat.

[38] Mr. Belch argued that Ms. Sutherland's evidence that there no buckle in the mat when the plaintiff fell should be accepted. He also argued that the video tape, which is objective and not subject to the frailties of human witnesses, shows no buckling in the mat before the plaintiff fell. Mr. Belch points to other causes which may have caused the plaintiff to fall. He referred to the plaintiff having previously injured her right knee when she fell at the Oxbury Mall in 1993 requiring arthroscopic surgery. He refers to the plaintiff having fallen in her driveway in March 2001 and her near fall on the bus in November 2001 and her having increasing pain in her left knee from that fall as current as three weeks before her fall in the market. He refers to the plaintiff having previous problems with her feet in the years past requiring her to wear orthotic devices in her shoes or boots and that at trial she could not recall whether she was wearing her orthotics on February 7th.

[39] In cross-examination, Mr. Belch referred the plaintiff to a physiotherapy report from St. Joseph's Hospital, where the plaintiff attended for therapy after her fall at the market. He

referred her to an entry made on the report dated April 30, 2002 under the heading Mobility/ Function/ Gait, which reads, "Wide base adopted, secondary valgus, deformities, knees, states tendency to catch right foot." He asked the plaintiff if she had said that to the physiotherapist and the plaintiff's reply was that she didn't remember if she said that because it was so many years ago. In my opinion, it would be unlikely the therapist would make such notation of something stated by the plaintiff unless she had said it.

[40] Ms. Edgars second factual basis regarding liability is founded on the evidence of Mr. Testolini and his evidence relating to the grate being imbedded in the floor of the vestibule with a border of cement being slightly higher than the floor. Ms. Edgar submits that in view of Ms. Sutherland's evidence that the plaintiff fell in that area the court should infer that the slightly raised cement border around the outside of the grate was the cause of the plaintiff's fall. Mr. Belch strongly objected to plaintiff's counsel advancing this alternative theory on the grounds that it was unfair to the defence. He pointed out that Ms. Edgar's opening address to the court set out the factual cause of the plaintiff's falls as a buckle in the mat. He also points out that the statement of claim pleads that the cause of the fall was a buckle in the mat. There is nothing pleaded about any irregularities in the level of the vestibule floor. Mr. Belch stated that had some fault in the floor been pleaded he would have engaged the services of professionally qualified people who would have provided specific measurements regarding the floor.

[41] Rule 25.06 (1) Provides that, "Every pleading shall contain a concise statement of the material facts on which the party relies for the claim or defence." The statement of claim pleads that a buckle in the mat caused the plaintiff to fall and that was the thrust of the plaintiff's evidence at trial. There was no request by the plaintiff to amend the statement of claim. With respect to pleadings I note the statement of Finlayson J.A. in *Kalkinis (Litigation Guardian of) v. Allstate Insurance Co. of Canada* (1998), 41 O.R. (3d) 528 (C.A.) at p. 533,

It has long been established that the parties to a legal suit are entitled to have a resolution of their differences on the basis of the issues joined in the

pleadings." If anything, the importance of proper pleadings is heightened on a motion for summary judgment and the matter should only be resolved as defined in the pleadings.

[42] I agree with the defendant that the secondary factual basis for liability argued by the plaintiff is not pleaded and not properly before the court. Quite apart from the propriety of the pleadings, Mr. Testolini said the cement edging was "pretty flush" with the floor and grating. Exhibit number 40 consists of two photographs taken of the vestibule in the spring of this year. The photos show the grate, the floor and the cement edge around the grate. From my observation of the photos, I would not dispute Mr. Testolini's comment.

Conclusions

[43] There are two likely explanations for the plaintiff's falling. One that she somehow tripped or stumbled on her own, the other that she tripped, as she described, on a buckle in the mat. In respect of whether she tripped on her own, upon entering the vestibule the plaintiff was walking on the smooth surface of the floor. The grates have a smooth surface. She then proceeded onto the mat, which from the photographs, appears to have a different surface texture. She has a history of falls. The evidence indicates that she was suffering increased pain in her left knee three weeks before her fall. Her boots were dry and her evidence was that the soles of the boots provided good traction. Although the plaintiff denies catching her foot or stumbling on the mat, I conclude that such an occurrence may have been the cause of her fall. If that was the cause of her fall, she was the author of her own misfortune and the defendant cannot not be held liable.

[44] The alternative theory is that a buckling in the mat caused her fall. There was a system or routine employed by the defendant to ensure the premises were reasonably safe for its patrons. Although the evidence shows that the system or routine was not regimented and supported by check lists showing the exact time that mats were checked, I accept the evidence of Mr. Testolini and Mr. Burgess that the system of inspection was continuous in respect of all aspects of the market including the vestibules. I accept their evidence and that of Ms. Hanwell that until the event of the plaintiff's fall, there had been no complaint about

the mats or of anyone having tripped on them. The mats were replaced with fresh mats weekly and sooner if required by weather conditions. The evidence of Mr. Testolini and Mr. Burgess was that the mats were placed over the grates as a safety measure to protect people from the risk presented by the grates when they were wet. On February 7th, the evidence indicates that it was not wet and there is no water on the floor of the vestibule. As I have stated earlier from my review of the video tape, numerous people traversed the vestibule and walked over the mat without difficulty with the last of such persons walking the length of the mat, without mishap, mere seconds before the plaintiff stepped on the mat.

[45] Ms. Edgar submits that the buckling seen in the video and photographs taken from it when the paramedics were attending the plaintiff shows the mat can buckle. I accept that submission. However, the circumstances are somewhat different. The paramedics were moving around and positioning a back board beneath the plaintiff, rather than simply walking the length of the mat. At the time the paramedics were present, the buckling in the mat developed, as seen from the time on the video, in a matter of seconds. In my opinion, if there was a buckling in the mat as described by the plaintiff when she tripped, although not seen by Ms. Sutherland, it similarly must have developed within a second or two since it is not seen in the immediately preceding frames of the video. The evidence indicates that there had been no previous problems experienced with the mats. Even if constant surveillance of the mat was being exercised at the time of the plaintiff's fall no response to such a condition would have been possible. As stated earlier in *Garofalo* and the cases cited therein, the law does not demand a system of constant surveillance and instant response. I find that the defendant had a system in place in regard to the mats in the vestibule and that in the circumstances it was a reasonable system, which provided that persons entering the market were reasonably safe.

[46] Having considered all the evidence and in particular the evidence pertaining to the vestibules and the mats, and the law as set out above I find that the plaintiff, upon whom the onus rests, has not established on a balance of probabilities that the defendant failed in its statutory duty to take reasonable care, considering in all the circumstances, to see that the

plaintiff was reasonably safe while on its premises. The defendant is entitled to a judgement dismissing the action.

Damages

[47] As a result of her fall the plaintiff received injuries to her head. She had a contusion on her forehead, and a bruise at the corner of her left eye. She hurts her knees. She suffered a small laceration to her left wrist and a bruise to her left forearm. Her right elbow was badly bruised and there was a bruise on her left shoulder. She said she suffered considerable pain from those injuries. As mentioned earlier, x-rays of her neck and wrist showed there were no fractures. She is still experiencing pain in the head and neck and her upper back and at trial she said those were her worst problems. The bruising cleared up in a month or two. The left wrist has recovered, her right elbow bothers her occasionally perhaps once a month and her knees have recovered.

[48] At trial the plaintiff was directed to her condition in the week following her fall. She said, "I was in horrible pain, I just laid in my bed, and my daughter took care of me, washed me, fed me, brought me drinks the water, and my husband." She said that because of her pain it took her about a half an hour to walk to the bathroom and back. However, there is a notation in the records of her psychologist Dr. Otchet dated February 11, 2002, that on February 9, 2002, two days after her fall, the plaintiff attended the funeral of her best friends' son.

[49] Prior to the accident the plaintiff had a long history of health problems. At seventeen she was diagnosed with depression and irritable bowel syndrome which conditions have continued to the present time. She last worked in 1984. At that time she was pregnant with her daughter. After the birth of her daughter she attended Fanshawe College but was required to drop out because of problems with a breast implant and had to under go three surgeries with respect to that. She has been in receipt of a CPP disability pension since 1983. In 1997-1998 Canada Pension did a follow up to determine if she was

still eligible for the disability pension and requested information from her then health care professionals. The plaintiff had been seeing a psychologist Dr. Otchet who wrote to CPP on February 27, 1998; "It is my impression that, from a psychological perspective, Mrs. George is unable to work in any capacity at the present time. Her long psychiatric history suggests that her problems are long-standing, and thus her prognosis is likely poor." Dr. Otchet prepared a psychological assessment report of January 15, 1998 in which she states her summary and conclusions of the plaintiff's condition at that time. She wrote, in part, "Mrs. George presents as a woman who is experiencing significant physical pain and dysfunction, as well as significant psychological distress including clinically significant levels of both depression and anxiety, all of which seem to be chronic problems for her". Dr. Otchet also mentions in her report that "Mrs. George has been involved with numerous mental health professionals since her teens."

[50] In a reassessment medical report to CPP dated September 23, 1997 Dr. Fodemesi wrote in part, "Significant stress in all spheres. The medical record I have has 250 pages of information." In a psychological assessment report of Dr. Crealock it is noted in part, "She has had panic attacks since 1991. Most recently she has begun to manage her panic by avoiding going to malls or crowded areas. Dr. Crealock's diagnosis was, Dysthymic Disorder-chronic depression; Panic Disorder with Agoraphobia and Parent- Child Relational Problem." The Oxford dictionary defines agoraphobia as an "extreme or irrational fear of open or public places".

[51] Dr. Tallon forwarded a report dated March 7, 1994 to CPP disability in which he noted the plaintiff problems at that time were; anxiety/depression/suicide attempts; migraine headaches; chronic vaginitis; bilateral foot pain; irritable bowel syndrome; multiple drug sensitivities; personality disorders/family dysfunction; back pain; environmental allergies and right knee strain (January, 1993). As mentioned earlier the plaintiff had fallen at the Oxbury Mall and injured her right knee which subsequently required surgery. The following are two excerpts from that report.

This woman has a long and complicated history of many physical and psychiatric problems. In my opinion, her major difficulty is her personality disorder which makes it very difficult for this woman to cope with any type of stress. She has many aches and pains, including migraines, abdominal pain (irritable bowel syndrome), and low back pain - all of which are affected by her inability to cope with stress.

.....

Things have not changed very much for Mary since this report was written. In my opinion, her multiple physical and emotional problems make her unemployable and my prognosis is that she will remain this way.

[52] In January 1991 and the plaintiff was subjected to a physical appraisal by an occupational therapist, Ms. Moira Sonnenberg. In her notes of the plaintiff's history the therapist wrote;

This 34-year-old female presents with an extensive medical history including irritable bowel, arthritis in the neck and shoulder, pelvic pain, and pain in the left pectoral area. On interviewing, the client states her primary problem is her "nerves". Mrs. George has not worked in six years but in the interim has attended Fanshawe College for period one year. The client is married (husband unwell), with an eight-year-old daughter.

[53] Under the heading of subjective findings, the therapist wrote;

Client states; her biggest problem is her nerves and irritable bowel. Physician has apparently discussed ankle fusion the client does not think she could take it. (R) foot pain radiating up to knee. Arthritic pain in neck and shoulders. Foot pain reduced by moving the joint around or applying a heating pad. No way of alleviating stomach pain. Husband helps with housework.

[54] In October 1991 Dr. Boyd saw the plaintiff at the general medicine clinic at Victoria Hospital. He noted his impression as follows:

This patient has a past history of somatoform disorder and some unspecified personality disorder. She seems to be very angry with the medical system and has unrealistic expectations of what can be done. Although her pain may be real to her, it seems to be minimal on exam.....

[55] The dictionary defines somatization as "the production of recurrent and multiple medical symptoms with no discernible organic cause".

[56] The plaintiff was seen in February / March 1998 by Dr. Bell a specialist in rheumatology. In his clinical records he wrote in part:

Mrs. George's problem started many years ago when she was investigated for severe irritable bowel syndrome. She was having severe diarrhea and incontinence with abdominal pain. Later on she developed musculoskeletal pain involving the neck, both shoulders, the arms, the legs and the back. This gave her poor interrupted sleep. She will wake up many times at night due to pain and she hardly get (sic) any sleep.

[57] Dr Bell diagnosed the plaintiff as having fibromyalgia in 1998. Dr. Bell saw the plaintiff again in 2001 in regard to her fibromyalgia. In a report of November 8, 2001 he wrote in part:

...She tells me that she has been having severe pain over the last three months and did not know any way to manage it. She has pain on a daily basis which is 10 out of 10 in severity. She tells me that she has been falling down.

A lot of her difficulty has been in her back and she has been receiving repeat facet injections, I assume, by Dr. Batorowics. These help for about six weeks.

She has all of the usual symptoms of fibromyalgia such as fatigue and non-restorative sleep, in addition to widespread pain everywhere.

She has been seeing Dr. Otchet, a clinical psychologist, for the last three years on a weekly basis. These sound like ventilation sessions. The psychologist has been trying to work with her on ways to cope with her chronic pain.

[58] Following her fall the plaintiff saw Dr. Batorowics at the pain clinic for the pain in her neck and her head. She had been treated in the past by Dr. Batorowics for abdominal pain. Following her fall, the plaintiff received injections in her neck and the back of her head from Dr. Batorowics which provided some relief for two to three months and then would have to be repeated again. She was treated by Dr. Batorowics from December 2002 till about November of 2005. In a consultation note dated December 6, 2002, Dr. Batorowics noted that the plaintiff's chief complaint was neck pain since her fall at the Covenant Garden Market in February 2002. A CAT scan had been done and the results were normal. Dr. Batorowics noted,

The patient complains of pain in the left base area of the skull, the left occipital area and the left neck, paracervical muscles and bilateral upper trapezius. The pain is worse when she bends her head forward and she maintains(sic) this position for some time after the accident being unable to extend her neck.

On occasion she at night has headaches and she wasn't able to sleep flat on the pillow, she had to maintain a semi-sitting position during the nights.

.....
On physical examination Mrs. George looks quite happy and range of motion of her neck is almost full except for turning toward the right gives her left sided discomfort especially pulling. Also, she has active trigger points in the left base of the skull, left paravertebral cervical muscles (local area) and upper trapezius.

.....
Impression: myofascial type of pain after whiplash type of injury.

[59] The plaintiff was seen again by Dr. Batorowics on June 4, 2004. At that time the doctor noted;

Today on physical examination she had slightly limited range of motion of the neck especially to rotation and left flexion with a tender point over the major occipital nerve, base of the skull and the T 4-5 supraspinal ligament and left trapezius.

[60] In a letter dated October 30, 2006 Dr. Batorowics wrote to the plaintiff's counsel, Ms. Edgar, responding to certain questions that Ms. Edgar had posed. The doctor was asked what her prognosis was with regard to the plaintiff and her reply was, "I cannot give you any binding time limit, it would be a speculation."

[61] Between July 2004 and October 2005 of the plaintiff attended upon Dr. Schulman, a chiropractor. She had numerous treatments relative to improving the range of motion of her neck. The plaintiff said that she derived some benefit from these treatments but they did not cure the pain in her neck.

[62] On August 30, 2005 Dr. John Clifford examined the plaintiff. In his consultation report he wrote in part,

Chief Complaint: this 49-year-old right-handed woman presents with complaints of severe pain in the back of her neck. The symptoms are associated with "crunching" when she turns her head and an inability to sleep more than 3 - 4 hours at night.

.....
Impression: the patient presents with widespread complaints of severe pain - in the absence of any obvious organic etiology.

[63] The patient was again referred to Dr. Clifford and in a report to the plaintiff's family physician dated March 7, 2006 Dr. Clifford wrote in part,

Unfortunately she continues to report "excruciating pain" involving the skull, neck, left shoulder and left arm (occasionally the right).....

.....
Accordingly from an organic perspective, residual complaints of pain would most likely be attributed to some combination of; Myofascial pain; Musculoskeletal deconditioning; Cervical spondylosis.

However, I would suspect the presence of significant inorganic factors, which include (sic) or all of the following: Psychogenic factors (e.g. anxiety, vigilance, somatization); Environmental contingencies (e.g., unresolved adversarial/entitlement issues); Dissimulation.

[64] Dr. Death saw the plaintiff at the St. Joseph's Hospital pain clinic on February 27, 2007. In regard to the plaintiff's complaint of head and neck pain the doctor expressed his impressions as follows:

In summary, Mrs. George is a woman who has suffered chronic pain from her fibromyalgia, but seems to have a new onset and a worsening of previously existing pain secondary to the countercoup injury she sustained in February of 2002 when she struck her head anteriorly on a large glass door and then on the posterior aspect when falling to the cement floor. Unfortunately, many psychosocial issues seem to be added to the chronicity and degree of suffering the patient has.

[65] At trial the plaintiff said that she suffered an increase in her fibromyalgia pain as a result of the fall. She said that by 2004 her fibromyalgia improved and returned to the level it had been before her fall. She also states that she now has had a dramatic improvement in

regard to the fibromyalgia. She is now able to sleep with the medication she has been prescribed. She also said that there is nothing that she absolutely cannot do as a result of the fall. However, as stated earlier she continues to complain of intermittent neck pain and continual head pain with intermittent headaches.

[66] The plaintiff has been under the care of three different psychologists sequentially and continuously since around 1988. They have been endeavoring to deal with her chronic pain and the other stresses in her life. The last and present psychologist treating the plaintiff is Dr. Iezzi. The plaintiff came under his care in October 2005. With the exception of one, all the interviews with the plaintiff were conducted by Dr. Iezzi's doctoral student, Jennie Ward. As appears to have been the case with the previous psychologists, the plaintiff has seen the Jennie Ward on a weekly basis.

[67] There are two notes made by Jennie Ward on October 12, 2005 and September 11, 2006 which indicates that the plaintiff becomes upset when told that her physical complaints do not have a physical cause. On October 12, 2005 Jennie Ward noted in the course of an interview with the plaintiff "she reports feeling frustrated following her appointment with Dr. Lefcoe, a psychiatrist, who reportedly labeled her as having "psychosomatic" pain. She is also angry with another nurse at one of the emergency departments in London who attempted to diagnose her as having Conversion disorder, according to Mary." On September 11, 2006 Jennie Ward noted, "She has also been particularly upset about a comment that her new family physician made about her pain having no physical cause."

[68] On July 19, 2006 Jennie Ward and Dr. Iezzi issued a report in respect to the plaintiff's psychological condition. The report is brief. It reads as follows.

Ms. George has been attending weekly individual psychotherapy sessions with myself at the Behavioral Medicine Service at the London Health Sciences South Street Hospital since October 5, 2006.(sic) (2005). Although she has a history of past psychological treatment, she has experienced a worsening in her psychological functioning subsequent to her fall at Covenant Garden Market in February, 2002. According to the DSM-IV-TR, the diagnostic manual for psychological disorders, she meets criteria for Post-dramatic Stress Disorder and Major Depressive Disorder. It is my opinion that Ms. George developed

Post-dramatic Stress Disorder as a result of her fall at the Covenant Garden Market. This fall also exacerbated a pre-existing depressive disorder.

[69] Dr. Iezzi is a clinical psychologist who specializes in the assessment and management of pain. He was called as a witness at the trial. He had previously met the plaintiff in 1991 as part of a therapy group. The plaintiff had been hospitalized due to a suicide attempt at that time. She had also been admitted to hospital in 2005 because of suicidal ideations and was referred to Dr. Iezzi. The plaintiff had also been hospitalized in 2006. Dr. Iezzi assigned the plaintiff to Jennie Ward, his doctoral student and it was she who conducted all the interviews with the plaintiff. From the initial interview in October 2005 Dr. Iezzi only spoke to the plaintiff once by telephone for about fifteen minutes. The report was prepared by Jennie Ward with the input and supervision of Dr. Iezzi. Having signed the report he considered he was responsible for it.

[70] He and Jennie Ward focused their attention to the plaintiff's fall in 2002. Dr. Iezzi said that the plaintiff had not told them of her previous falls. They did not perform research such as would be done in preparing a medico-legal report. Their resources were the plaintiff's hospital charts and what the plaintiff told them. Dr. Iezzi described what he called a 'complex' post traumatic stress disorder, (PTSD) and compared it with a "circumscribed" PTSD. A complex PTSD may contain a constellation of things which could produce PTSD. He and his student focused only on the issue of the plaintiff's fall and did not take into account all the past history of the plaintiff. Accordingly, he confirms a "circumscribed" PTSD arising from the plaintiff's fall. The PTSD found by Dr. Iezzi was that the plaintiff had developed a fear regarding attending the Covent Garden Market. They prepared the plaintiff to make an attempt to revisit the market but it was unsuccessful. The plaintiff said at trial that she attended the market and began to sweat and cry and could not go in.

[71] Mr. Belch asked if the doctor was familiar with the plaintiff's having problems with the medical community. Dr. Iezzi said,

There seems to be a pattern of where she was not taken very seriously by medical assessors and treaters, which being a pain psychologist, that's not

unusual in terms of, by the time patients come to me, this would be a fairly common refrain.

[72] Dr. Iezzi was referred to an adult psychiatry report dated June 16, 2005, prepared by a clinical clerk of Dr. Lefcoe, who is a psychiatrist. A portion of the summary of that report reads;

Mrs. George is a 49-year-old female who unfortunately suffers from many significant medical problems she also suffers from Axes 1 disorders of dysthymia, major depression and a possible component of PTSD given the history of physical abuse.....

[73] Dr. Iezzi was aware of the plaintiff's history of abuse. In respect of the above quote he said;

Again, I can't speak for the resident, but the P.T.S.D associated with physical abuse would be consistent with a history of having complex post-traumatic stress disorder, which is a term we use to describe people who have a history of multiple traumas over the course of a lifetime..... Different from a circumscribed P.T.S.D that she has associated specifically with the event of having a fall in 2002.

[74] Dr. Iezzi further commented;

If she had not fallen that day, she would not have a circumscribed P.T.S.D. associated with falling in the Covent Market. She wouldn't have all of the psychological reactivity, she wouldn't have all those thoughts, she wouldn't have the anxieties about avoiding the market, she wouldn't be afraid of carpets. Yes, she would still have some P.T.S.D. associated with other traumas in the past.

.....
...you will see that, particularly in the last year, much of the content focused on that circumscribed P.T.S.D. We did not focus on physical abuse as a child. We did not focus on those other traumatic events that occurred in the distant past.

[75] In regard to his prognosis of the plaintiff's circumscribed PTSD Dr. Iezzi said;

..... it would be guarded. At this point, even though she has improved, she has improved from when she first came to us, I still would see some of the progress as somewhat fragile and it remains to be determined whether she'll be able to

maintain the newly learned skills. I guess I could add that another stressor could come along and knock her back down to square one.

.....

Well, to go back to the word "circumscribed," it keeps her from going to the Covent Market, she tends to stay away from even places that have carpeting, she's much more leery about having another fall, much more cautious and particularly, when she's out of the house and in the open space, and it still produces some anxiety whenever she is exposed to cues associated with that event.

[76] In *Athey v. Leonati* , [1996] 3 S.C. R. 498 the Supreme Court of Canada stated the following in respect of compensation in tort law.

¶ 32 To understand these cases, and to see why they are not applicable to the present situation, one need only consider first principles. The essential purpose and most basic principle of tort law is that the plaintiff must be placed in the position he or she would have been in absent the defendant's negligence (the "original position"). However, the plaintiff is not to be placed in a position better than his or her original one. It is therefore necessary not only to determine the plaintiff's position after the tort but also to assess what the "original position" would have been. It is the difference between these positions, the "original position" and the "injured position", which is the plaintiff's loss.....

[77] Prior to the plaintiff's fall she had for many years suffered chronic pain syndrome and fibromyalgia. The foregoing excerpts from medical records prior to the fall show she suffered from many problems, including complaints of pain in her neck and shoulders. She complains of that presently. She currently complains of a constant pain in her head. She complains of intermittent headaches, which at times are severe, however, the records show that she previously complained of migraine headaches. The plaintiff said the current headaches are different from the previous migraines. She has continually suffered from depression both before and after her fall and has been treated continuously, since 1997-98 by a series of psychologists in regard to her chronic pain. From the various psychologists record it is apparent that the plaintiff has had problems in her relationship with her daughter, particularly in the daughter's teenage years. She has experienced problems in her relationship with her mother and her husband. She had a history of suicidal ideation prior to her fall as well as

afterwards. She has been subject to many stresses in her life, with which she was unable to cope. Further, from the above medical report excerpts and in particular the report of Dr. Clifford, the medical profession has been unable to find an organic basis for her complaints.

[77] Her present complaints are that of pain in her neck, headaches and a constant pain in the back of her head and pain in her upper back. Her fibromyalgia was exacerbated by the fall. It returned to its previous level around 2004 and she has since experienced a dramatic improvement with it. Dr. Iezzi diagnoses an exacerbation in her depression as a result of the fall. He also states that she has shown improvement. He diagnoses the plaintiff suffering from a PTSD of a nature which prevents her from attending at the market and being more anxious and cautious when outside of the house. On the other hand, the plaintiff had previously been diagnosed as suffering from agoraphobia. The evidence suggests that the plaintiff may also be suffering from PTSD from physical abuse during her childhood. Dr. Batorowics' impression was "myofascial type of pain after whiplash type of injury". Dr. Death commented in February 2007, that, "Unfortunately, many psychosocial issues seem to be added to the chronicity and degree of suffering the patient has".

[78] Previously the plaintiff and her husband shared the mowing of their very large back yard lawn. Presently the plaintiff does that alone although she has to pace herself. As mentioned earlier, the plaintiff said that there was nothing that she is absolutely unable to do because of her fall, although, she does have to pace herself.

[79] Keeping in mind, as stated in *Athey* that, "It is the difference between these positions, the "original position" and the "injured position", which is the plaintiff's loss", I assess the plaintiff's damages for pain and suffering and loss of enjoyment of life, at \$60,000.00. I also allow the special damages claimed, namely OHIP at \$1,628.05 and the plaintiff's account for her chiropractor in the amount of \$ 1,726.25 for a total of \$63,354.30.

[80] In respect of the FLA claim of the plaintiff Crystal George, she did not testify, apparently on the directions of her doctor. She is pregnant. The only evidence in regard to

what she did for the plaintiff was that she stayed at the plaintiff's home for three weeks and fed, washed and tended to her. I assess Crystal George's services at \$1500.00. Because of the long standing problems in the relationship between mother and daughter I would not award any assessment for loss of care, guidance and companionship.

[80] In regard to the husband's FLA claim, he was not able to testify because of his severe health problems which have become worse since 2001. He suffers from diabetes, a heart condition and as the plaintiff described "dementia". He has severe problems with short term memory, for example, the plaintiff might find him eating and have to remind him that he had already just eaten his lunch. He cannot go out alone. Following the plaintiff's fall he assisted her by helping her get to the bathroom and bring her food and drink along with their daughter and attended to the house work for a time. There is no direct evidence that because of the plaintiff's fall Wilson George was neglected by the plaintiff. In fact the plaintiff looked after him, notwithstanding her own health problems. She said that she would have liked to have been able to do more. The deteriorating health of Wilson George added considerable stress to the plaintiff life causing her to feel overwhelmed and in 2005-06 she considered leaving the marriage. On the limited information available in respect of Wilson George's claim I assess his FLA claim for services rendered to the plaintiff and for loss of care guidance and companionship at \$5,000.00

[81] In summary, I assess the plaintiff's total damages including her out of pocket expenses at \$63,354.30. The FLA claim of Crystal George is assessed at \$1500.00 and that of Wilson George at \$5000.00. The plaintiffs' action, however, is dismissed. If counsel are not able to agree in respect of costs they may make written submission within 30 days

RELEASED: July 25, 2007

"Justice K.F. Ross"

Justice K.F.Ross

